

QRC 19.2 CARE UNDER FIRE

Return Fire As Directed/Required Before Providing Any Medical Treatment

1. Direct casualty to remain engaged as a combatant (if possible)
2. Direct casualty to move to cover and apply self-aid, if able
3. If casualty cannot return fire, have casualty “play dead” until enemy fire is suppressed and rescue plan is developed and executed
4. If the casualty has life-threatening bleeding coming from a limb, apply a tourniquet on the limb that is bleeding
5. Evacuate

QRC 19.3 COMBAT APPLICATION TOURNIQUET (C-A-T®) -CARE UNDER FIRE-

Always Treat for Shock, see QRC 19.9

Use Personal Protective Equipment (Gloves)

WARNING: Do not use wire, shoestring, or anything less than 1 inch wide.

WARNING: Never loosen or remove tourniquet once it is applied.

1. Expose wound, and place C-A-T® around extremity as high and tight as possible
2. Route strap through friction adapter buckle
3. Pull free end of band tight, and secure it onto itself using the velcro
4. Twist the windlass rod until bleeding stops

NOTE: If you can twist the windlass rod more than three times, start over and ensure that the band is pulled tight against the extremity before twisting the windlass rod.

5. Insert rod into windlass clip
6. Pull windlass strap tight over windlass clip and band to prevent the windlass rod from unwinding
7. Mark the patient's head with the letter “T” and time that the tourniquet was applied, also annotate the date/time/location of the C-A-T® on the casualty card
8. If needed, apply second tourniquet below initial tourniquet if bleeding has restarted or not stopped

QRC 19.4 CONTROL BLEEDING -TACTICAL FIELD CARE-

Always Treat for Shock, see QRC 19.9

Use Personal Protective Equipment (Gloves)

1. Apply direct pressure with hand and use dressing if available
2. If bleeding has not stopped with direct pressure, use pressure points with elevation to control the bleeding
3. If available, use combat gauze

NOTE: If the bleed is coming from groin or armpit, where tourniquet cannot reach, use combat gauze with direct pressure only after you move to tactical field care phase.

4. If bleeding has not stopped with direct pressure, pressure points, or elevation, apply a tourniquet to stop bleeding in the tactical field care/conventional phase.
5. Document all treatment on DD Form 1380.
6. Evacuate as needed

**QRC 19.5 COMBAT APPLICATION TOURNIQUET
(C-A-T®) -TACTICAL FIELD CARE-**

WARNING: *Do not* use wire, shoestring, or anything less than 1 inch wide.

WARNING: *Never loosen or remove* tourniquet once it is applied.

NOTE: Leave tourniquet area exposed for quick visual reference.

- Expose wound, and place C-A-T® around extremity 2-4 inches if possible
- Route strap through friction adapter buckle
- Pull free end of band tight, and secure it onto itself using the Velcro®
- Twist the windlass rod until bleeding stops

NOTE: If you can twist the windlass rod more than three times, start over and ensure that the band is pulled tight against the extremity before twisting the windlass rod.

- Insert rod into windlass clip
- Pull windlass strap tight over windlass clip and band to prevent the windlass rod from unwinding
- Mark the patient's head with the letter "T" and time that the tourniquet was applied and annotate the date/time/location of the C-A-T® on the casualty card
- If needed, apply second tourniquet below initial tourniquet if bleeding has not stopped or restarted

QRC 19.6 AIRWAY

Determine if Care Under Fire, see QRC 19.2 or
Tactical Field Care, see QRC 19.4
Always Treat for Shock, see QRC 19.9
Use Personal Protective Equipment (Gloves)

HEAD-TILT/CHIN-LIFT METHOD

(No head/neck/back injuries)

1. Place one hand on the casualty's forehead
2. Place fingertips of other hand under bony aspect of casualty's chin
CAUTION: Do not press on the soft area under the chin, doing so may block the casualty's airway.
3. Gently tilt casualty's head by applying downward pressure on forehead while lifting chin upward
4. The mouth should open; use your thumb to depress the casualty's lower lip slightly to keep the mouth open

JAW-THRUST METHOD

(Possible head/neck/back injuries)

1. Take a position at the top of the casualty's head, resting your elbows on the ground on either side of the head
2. Reach forward and place one hand on each side of the casualty's lower jaw, at the angle (just below the hinge) of the jaw
3. Stabilize the casualty's head between your forearms
4. Using your fingers, gently push the lower jaw forward to open the airway
5. The thumbs can be used to depress the casualty's lower lip slightly to keep the mouth open

CAUTION: Do not tilt or rotate the casualty's head, doing so may cause further injury to the spinal cord.

6. Immobilize the casualty using any means possible (e.g., cervical collar, rolled up towels, boots filled with sand) (QRC 19.10)

QRC 20.1 MEDICAL EVACUATION (MEDEVAC) 9-LINE

LINE 1—Location, give grid coordinates _____

LINE 2—Radio frequency and call sign _____

LINE 3—Number of patients by precedence (if two or more categories must be reported, say “break” between) _____

A—Urgent (Immediate, ASAP)

B—Priority

C—Routine

D—Convenience

LINE 4—Special equipment required _____

A—None

B—Hoist

C—Evacuation equipment (Stokes® litter, etc.)

D—Ventilator

LINE 5—Number of patients by type _____

A—Litter

B—Ambulatory

C—Escort

LINE 6—Security of pickup site (Wartime) _____

N—No enemy troops in area

P—Possible enemy troops in area (approach with caution)

E—Enemy troops in area (approach with caution)

X—Enemy troops in area (armed escort required)

*Number and type of wound, injury, illness (Peacetime)—Specific info (gunshot, shrapnel, bleeding) _____

LINE 7—Method of marking pickup site _____

A—Panels

B—Pyrotechnic signal

C—Smoke signal

D—None

E—Other (mirror, person, light, etc.)

LINE 8—Nationality and status of casualties _____

A—US military

B—US civilian

C—Non-US military

D—Non-US civilian

E—Enemy prisoners of war (EPW)

LINE 9—Status of chemical, biological, radiological, and nuclear contamination (Wartime) _____

C—Chemical

B—Biological

R—Radiological

N—Nuclear

QRC 7.1 WEAPON SAFETY

- Treat all weapons as loaded.
- Never point a weapon at anything you do not intend to shoot.
- Keep your weapon on SAFE until you are ready to fire (unless directed otherwise).
- Keep your finger off the trigger until sights are on target and you are ready to fire.
- Positively identify your target, be aware of what is around and behind the target.
- Never engage in horseplay while handling weapons.

QRC 14.1 5-Cs UXO/IED BATTLE DRILL

5-Cs	Reaction
Confirm	the unexploded explosive ordnance (UXO).
Clear	the area 300 meters (1,000 feet) around the UXO and utilize all frontal and overhead protection and report the UXO to the operations center (see paragraph 14.8).
Cordon	the perimeter around the UXO.
Check	the immediate area for other UXOs. Perform 5/25/Sky meter check.
Control	the site access, maintain security.

QRC 14.2 EOD 9-LINE REPORT

LINE 1—Date-Time Group (when the item was discovered): _____

LINE 2—Report Activity and Location (unit ID and grid location of the IED/UXO): _____

LINE 3—Contact Method:

Radio Frequency: _____

Call Sign: _____

Point of Contact: _____

Telephone Number: _____

LINE 4—TYPE of IED/Ordnance (unknown, dropped, projected, placed, thrown, UAS rotary or fixed wing, or possible IED). Describe the IED/UXO: _____

LINE 5—CBRN Contamination (Negative or Positive)

If present, describe (be as specific as possible): _____

LINE 6—Target/Resources Threatened:

Personnel (coalition forces, host nation, civilian): _____

Facilities: _____

Equipment: _____

Other Assets: _____

LINE 7—Impact of Mission (short description of current tactical situation and how the IED/UXO affects the status of the mission): _____

LINE 8—Protective Measures/Evacuation (measures taken to protect or evacuate personnel and equipment): _____

LINE 9—Recommended Priority:

(Immediate)

(Indirect)

(Minor)

(No Threat)

QRC 14.3 DROPPED ORDNANCE (1 of 2)



Magazine shown for size reference.

QRC 14.3 DROPPED ORDNANCE (2 of 2)



Rifles shown for size reference.

QRC 14.4 PROJECTED ORDNANCE



Rifle(s) shown for size reference.

QRC 14.5 THROWN/PLACED ORDNANCE (1 of 2)



Magazine shown for size reference.

QRC 14.5 THROWN/PLACED ORDNANCE (2 of 2)



Magazine(s) shown for size reference.

QRC 15.1 NINE PRINCIPLES OF IED COMBAT

1. Maintain an offensive mindset.
2. Develop and maintain situational awareness.
3. Remain observant.
4. Avoid setting patterns.
5. Maintain standoff.
6. Maintain 360-degree security.
7. Maintain tactical dispersion.
8. Utilize blast and fragmentation protection.
9. Know and use technology.